



Policy

Medicines & Therapeutics Formulary

Reference #: HMP07

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All staff

Introduction

The [Mandatory Policy State-wide Medicines Formulary MP77/18](#) describes the overarching governance process for medications in Western Australian (WA) public hospitals. Medications that are not listed on the formulary require an approval and oversight process to ensure safe and judicious use.

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Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGH Mission & Values	☐	Other:	☐

Policy Principles

To detail the governance process for medications able to be prescribed, dispensed, supplied and administered to patients within SCGOPHCG.

1.0 Roles and Responsibilities

Medicines and Therapeutics Committee (MTC)	The MTC is responsible for providing overall governance of medicines management at SCGOPHCG to ensure the appropriate, efficacious, safe and cost-effective (quality) use of medicines and therapeutics.
MTC Chair	The MTC Chair is responsible for reviewing all IPA applications.
Director Clinical Services (DCS)	The DCS is the Executive Sponsor of the MTC. They are responsible for reviewing all IPA requests costing greater than \$50,000 per annum. They are the delegated signatory for all Medication Access Programs (MAP) and must approve a MAP before it is commenced.
Heads of Department (HODs)	HODs are responsible for reviewing all IPA requests from their department.
Medical Co-Directors	Medical Co-Directors are responsible for reviewing all IPA requests costing greater than \$10,000 per annum, from their respective division
Pharmacists	Pharmacists are responsible for making recommendations in line with the SMF.
Prescribers	Prescribers are responsible for complying with SMF policy when initiating medications for WA public hospital patients. When requesting an IPA, the prescriber is responsible for providing a true, comprehensive and accurate history of the evidence and clinical details supporting non-formulary use of the medication.
High-Cost Medicines (HCM)	Are medicines that meet or exceed a specified monetary threshold, which is established by the MTC. This threshold ensures that the most expensive treatments undergo a comprehensive and rigorous assessment, taking into account both clinical efficacy and fiscal responsibility. For the purposes of this document the threshold is set at \$200,000 per annum and must include explicit criteria to indicate when therapy is no longer warranted for the patient.

2.0 Statewide Medicines Formulary (SMF)

The SMF is a list of medications with approved indications overseen by the WA Therapeutic Advisory group (WATAG). The purpose of the SMF is to deliver optimal

patient outcomes in an equitable manner through a single list of approved medicines for the WA health system which is evaluated, implemented, and managed in a state-wide approach. It is underpinned by the Mandatory Policy State-wide Medicines Formulary MP77/18.

The SMF can be accessed via the [FormularyONE](#) website.

3.0 National Blood Authority (NBA)

Federally funded immunoglobulin products are available via the NBA and supplied by Pathology Departments. Approved indications and eligibility information can be accessed at <https://www.criteria.blood.gov.au>

For blood products used for Non-NBA indications, an Individual Patient Approval from the MTC is required. The cost of supply will then be borne by SCGOPHCG.

4.0 Medicines and Therapeutics Committee and non-formulary prescribing

The MTC is responsible for providing governance and oversight of medicines at SCGOPHCG. This includes the review, approval and oversight of non-formulary medication use.

Prescribing and Access for non-formulary medications can occur via two pathways, both requiring approval from the MTC prior to treatment initiation:

- 1) Individual Patient Approval (IPA), or
- 2) Medication Access Programs

The following therapeutics are eligible for an IPA or MAP application:

- A registered medicine with the Australian Therapeutic Goods Administration (TGA) or medicines not registered by the TGA, but available under the TGA Special Access Scheme (SAS).
- Unscheduled medicines marked or generally accepted as a medicine e.g. Total Parenteral Nutrition (TPN), dialysis fluids
- Blood products that do not meet National Blood Authority (NBA) criteria

5.0 Individual Patient Approvals (IPA)

The IPA process is a mechanism to allow access to a medication or blood product that is not accessible via the SMF or NBA. The IPA process is in place to ensure that access to medicines is equitable, medication usage is evidence based, patient safety is assured, and financial sustainability is maintained. Overarching responsibility for the oversight of the process is vested in the MTC.

Individual Patients Approvals are for:

- Medications not listed on the SMF or,
- Medications used for an indication that is not approved by the SMF or,
- Blood Products for an indication not approved by the NBA.

5.1 Applications

Only prescribers employed by SCGOPHCG may make IPA applications.

Informed Consent should be obtained as per the [WA Health Consent to Treatment Policy OD0657/16](#), Ethical and Judicious use should be consistent with the guiding principles of

the Council of Australian Therapeutic Advisory Groups ([CATAG Guiding Principles for the quality use of off-label medicines](#)).

IPA applications must be submitted electronically using the [WA Individual Patient Approval System \(WAIPAS\)](#) available via [FormularyONE](#). Treatment cannot be initiated until the application is approved.

All IPAs require endorsement from the relevant medical specialty Head of Department.

Application should be made either by the treating consultant, or another member of the medical team “on behalf of” the treating consultant included in the application.

In the instance where an IPA applicant is also the Head of Department, approval must be granted by a nominated senior colleague.

5.2 Cost of Treatment

Different cost of treatment per patient per year will have different approval requirements as defined in Table 1.

All applications are costed to the division for which the patient was admitted for the relevant episode of care.

Cost patient / year	Medical / HoD	MTC Chair	Medical Co-Director	MTC Committee	Director of Clinical Services	Director of Finance	Executive Director
\$0 – \$10,000	Yes	Yes					
\$>10,000 – \$50,000	Yes	Yes	Yes				
\$>50,000 – \$200,000	Yes	Yes	Yes	Yes [#]	Yes		
> \$200,000	Yes	Yes	Yes	Yes [#]	Yes	Yes	Yes [*]

Table 1: Approvals of different costs of treatment.

[#]MTC Committee review is required for all non-urgent IPA reviews for medicines above \$50,000

All IPAs with a cost of > \$200,000 per patient per year must be accompanied by a High Cost Medicines form which will be prepared by the Senior Pharmacist (Formulary & Governance).

^{*}Executive Director approval is only required for the initial IPA. Requests for the continuing supply of the IPA can be authorised by the Director of Clinical Services.

5.3 Assessment

Applications will be assessed according to the cost of medication, the setting of use, and the urgency. An application can be considered urgent where a delay in treatment could be life threatening and/or may cause significant morbidity/mortality. Any assessor(s) may identify circumstances where broader consultation needs to be sought to best assess the merit, context and clinical impact of the application.

5.4 Notification of Decision

Applicants will be notified of the outcome of any assessment via email subsequent to the review.

5.5 Appeal of a decision

If an application is not approved, the applicant may lodge an appeal via WAIPAS. The application may be referred to the Director of Clinical Services for escalation as required. Appeal or review of a decision is able to be sought by an applicant where:

- Additional evidence is able to be provided to support the application.
- An applicant believes that the review process misinterpreted the request or evidence as a basis for denial.
- An applicant believes the denial of treatment will cause a significant risk of mortality/morbidity.
- Other reasons apply as agreed by the Director of Clinical Services.

5.6 Pre-approved Medications and conditions

The MTC permits exceptions to the standard IPA process for certain low-cost, low-risk and high-usage medicines. This approach reduces administrative workload for clinicians while ensuring compliance with the WA Statewide Medicines Policy. Medications listed in Appendix A will receive automatic IPA approval, as they have been evaluated as safe, clinically appropriate, and cost-effective for their specified conditions.

- IPA applications must still be submitted by the requesting clinician via WAIPAS.
- The Senior Pharmacist (Formulary & Governance) will complete endorsements on behalf of the Head of Department and MTC Chair.
- Treatment may commence without waiting for approval to be granted via WAIPAS.

5.7 Urgent IPA requests

During standard business hours the requesting clinician should contact the MTC Senior Pharmacist (Formulary & Governance) for urgent out of session review by the MTC Chair.

Outside of standard business hours, if the application cannot wait until the next business day, treatment may be supplied by the SCGH Pharmacy Department provided a review of clinical efficacy and safety has been documented.

- If the cost of the medication supplied is less than \$5000, contact the relevant pharmacist (late shift, weekend or on-call) to facilitate supply.
- If the cost of the medication supplied is more than \$5000, the on-call executive must be contacted for verbal approval and this must be documented in the patient's medical record.

The application must still be lodged via WAIPAS for retrospective review and approval.

5.8 Outcome Reporting

- Outcome reports are required to inform future IPA requests and inform submissions to WAMEP for the Statewide Medicines Formulary.
- The requesting clinician is required to submit the outcome report as stipulated in the approval confirmation, prior to the end date of the IPA approval.
- The MTC is not responsible for requesting outcome reports. Delays in providing outcome reports may impact future IPAs considered by the MTC.

6.0 Medication Access Programs

MAPs refer to any arrangement where a medication is offered for free or at a reduced cost to SCGOPHCG, outside the regular process and governance structures for pharmaceutical procurement. MAP applications are classified as one of the following:

- **Compassionate Use / Access (CA):** A program offered by a pharmaceutical sponsor to provide a medicine free of cost for indications that are not already included in a funded scheme. Compassionate use may be determined on an individual patient basis or as part of a wider program.
- **Expanded Access Programs (EAP):** A program offered by a pharmaceutical sponsor that provides an investigational product free of charge when associated with participation in a clinical trial.
- **Cost-share Programs (CSP):** A program offered by a pharmaceutical sponsor that offers a medication commercially at a reduced price. Treatment costs are shared between a pharmaceutical sponsor and Health Service Provider (HSP)
- **Product Familiarisation Program (PFP):** A program offered by a pharmaceutical sponsor that is designed to allow the prescriber to evaluate and become familiar with a treatment while PBS listing is being finalised / sought.

Before treatment can be initiated, the MAP requires approval from the following stakeholders:

- Relevant HoD
- Relevant Pharmaceutical Sponsor
- SCGOPHCG Pharmacy Department
- MTC
- DCS

7.0 Compliance/Performance Monitoring

- Outcomes and decisions are to be tabled monthly in the MTC Committee meeting with decisions recorded for each application
- The MTC provides a quarterly report to the Clinical Governance Committee including all non-formulary prescribing approvals.
- Non-compliance with this procedure should be reported to the Medicines and Therapeutics Committee (MTC) and the Director of Clinical Services who is responsible for escalating instances of non-compliance.
- All de-identified IPA approvals are accessible to the WATAG via WAIPAS.

8.0 Treating Serious Adverse Events (SAE) Resulting from Clinical Trials

SCGOPHCG prescribers are to follow the Formulary Management Procedure when treating SAEs that may have been caused by administration of clinical trials (CT) investigational products. The SAEs treatment applies to both medicines and blood products, which may require IPA (refer section 5.0 above).

To avoid delays related to treatment and to optimise patient outcomes, all CT patients admitted to SCGOPHCG for treatment of SAEs will follow standard SCGOPHCG procedures. Note that Clinical Trials Pharmacy:

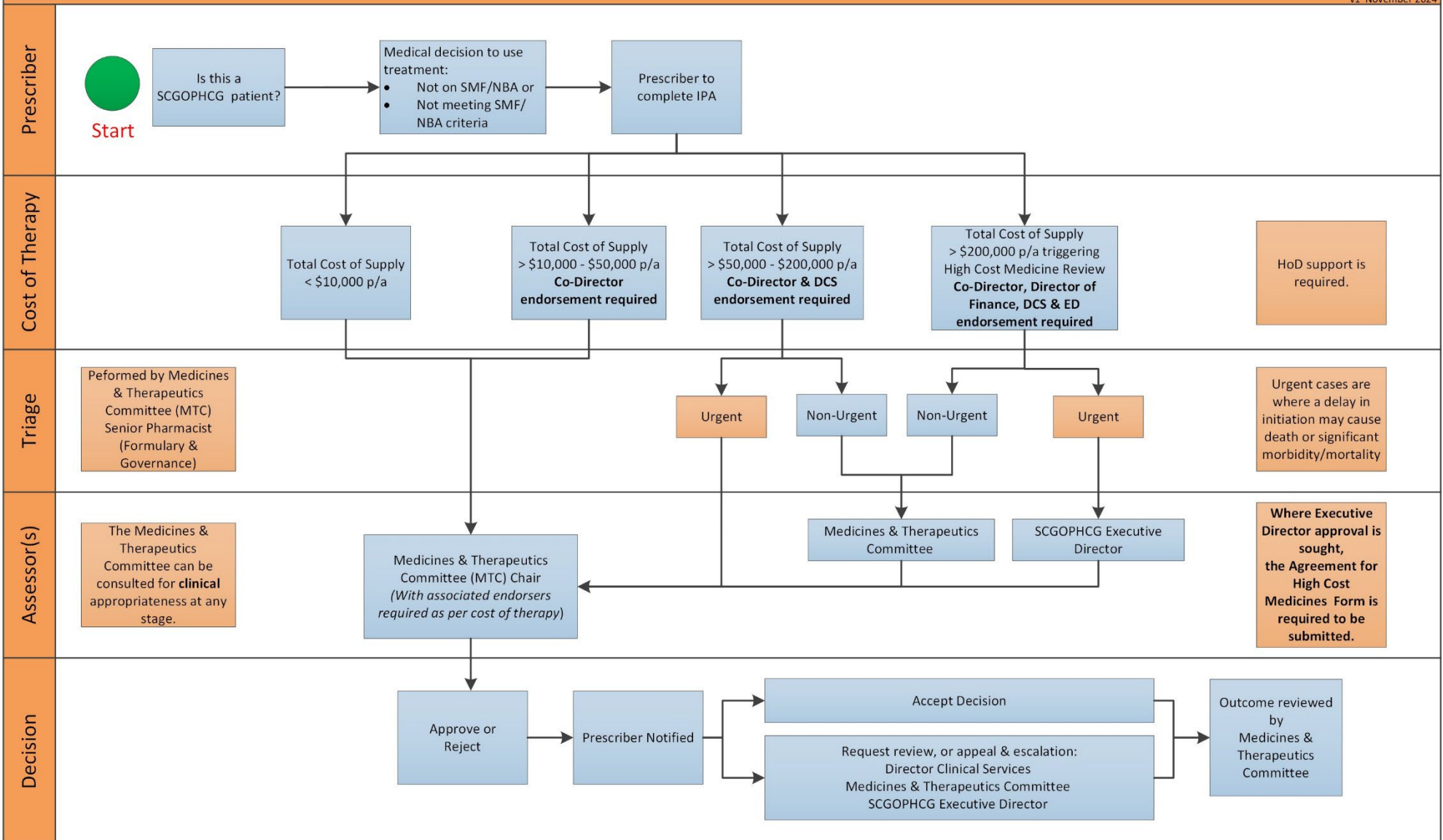
- Will not be accepting/storing CT specific rescue medicines supplied by sponsors to treat the CT indicated SAEs for their CT patients. These patients will be treated with hospital stock and hospital procedures will be followed.

If CT sponsors will reimburse the cost of SAEs treatment, the respective CT study team or Business Managers of the costed team will be responsible for seeking reimbursement.



Sir Charles Gairdner Osborne Park Hospital Care Group – Individual Patient Approval (IPA) Pathway

v1 November 2024





10.0 References & Related Documents

- [SCGOPHCG Medicines Management Policy and Procedure](#)
- [SCGH MMP: Medication Access Programs \(MAP\)](#)
- [MP 0077/18 Statewide Medicines Formulary Policy](#)
- [Western Australia Statewide Medicines Formulary Governance and Procedures](#)

Authorisation & Version Control

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

Appendix A

Name	Approved indication(s)
Melatonin oral preparations	• Insomnia/ sleep disorder for inpatient use only Standard treatment doses apply.